Clinical Study Summary Report (CSSR) Document Status: Draft | **Version:** 0.1 | **Date:** 06-Apr-2026

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1. Administrative & Study Identification

Full Study Title: A Study to Learn About the Effects of the Combination of Elranatamab (PF-06863135), Daratumumab, Lenalidomide or Elranatamab and Lenalidomide Compared With Daratumumab, Lenalidomide, and Dexamethasone in Patients With Newly Diagnosed Multiple Myeloma Who Are Not Candidates for Transplant **Short Title/Acronym:** MagnetisMM-6 **Protocol Number:** PF-06863135 **NCT Number:** NCT05623020 **Sponsor/Company Name:** Pfizer **Investigational Product:** Elranatamab (BCMA-CD3 bispecific antibody) in combination with Daratumumab and Lenalidomide or Elranatamab and Lenalidomide **Phase of Development:** Phase 3 **Medical Monitor:** Pfizer Clinical Operations/Medical Monitor **Trial Status:** Recruiting

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2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: There are 2 parts to this study. Part 1 will characterize the safety and tolerability of elranatamab in combination with daratumumab and lenalidomide or in combination with lenalidomide and will identify the optimal dose(s) of the combination regimen. Part 2 of the study will evaluate the minimal residual disease (MRD) negativity rate and the progression-free survival (PFS) of the combination of elranatamab, daratumumab, and lenalidomide or elranatamab and lenalidomide compared with the combination of daratumumab, lenalidomide, and dexamethasone in participants with newly diagnosed transplant-ineligible multiple myeloma. **Secondary Objectives:** To assess overall survival, overall minimal residual disease negativity rate, and sustained MRD negativity rate (Part 2).

2.2 Background & Rationale To address the need for more effective upfront therapies in patients with newly diagnosed multiple myeloma (NDMM) who are transplant-ineligible. Elranatamab is a bispecific antibody: binding of elranatamab to CD3-expressing T-cells and BCMA-expressing multiple myeloma cells causes targeted T-cell-mediated cytotoxicity. Introducing T-cell redirection therapy in the first-line setting leverages a healthier, intact immune system, potentially yielding superior and more sustained disease eradication without the need for stem cell transplantation.

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3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Active Comparator (Daratumumab + Lenalidomide + Dexamethasone) **Blinding:** Open-label **Randomization:** Randomized (2-Arm Phase 3) **Status:** Recruiting

3.2 Planned Enrollment & Duration Target N: 966 evaluable patients **Number of Sites:** Multicenter, Global (including Australia, Canada, China, France, Germany, Japan, Spain, USA, etc.) **Estimated Study Start:** Nov-2022 (Based on NCT registration) **Estimated Primary Completion:** Active enrolling/Ongoing

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4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Diagnosis of multiple myeloma (MM) as defined by IMWG criteria. 2. Measurable disease based on IMWG criteria as defined by at least 1 of the following: - Serum M-protein ≥ 0.5 g/dL; - Urinary M-protein excretion ≥ 200 mg/24 hours; - Involved FLC ≥ 10 mg/dL (≥ 100 mg/L) AND abnormal serum immunoglobulin kappa to lambda FLC ratio (<0.26 or >1.65). 3. Part 1: Participants with relapsed/refractory multiple myeloma (RRMM) who have received 1-2 prior lines of therapy including at least one immunomodulatory drug and one proteasome inhibitor: or participants with newly-diagnosed multiple myeloma (NDMM) that are transplant-ineligible as defined by age ≥ 65 years or transplant-ineligible as defined by age < 65 years with comorbidities impacting the possibility of transplant. 4. Part 2: Participants with newly-diagnosed multiple myeloma that are transplant-ineligible as defined by age ≥ 65 years or transplant-ineligible as defined by age < 65 years with comorbidities impacting the possibility of transplant. 5. ECOG performance status ≤ 2. 6. Not pregnant and willing to use contraception. 7. For participants with RRMM: Resolved acute effects of any prior therapy to baseline severity or CTCAE Grade ≤ 1.

4.2 Exclusion Criteria 1. Diagnosis of Smoldering Multiple Myeloma, Monoclonal gammopathy of undetermined significance (MGUS), Waldenström's Macroglobulinemia, or Plasma cell leukemia. 2. Active, uncontrolled bacterial, fungal, or viral infection, including (but not limited to) COVID-19/SARS-CoV-2, HBV, HCV, and known HIV or AIDS-related illness. 3. Any other active malignancy within 3 years prior to enrollment, except for adequately treated basal cell or squamous cell skin cancer, carcinoma in situ, or Stage 0/1 with minimal risk of recurrence per investigator. 4. For participants with RRMM: Previous treatment with a BCMA-directed therapy or anti-CD38-directed therapy within 6 months preceding the first dose of study intervention in this study. Stem cell transplant ≤ 3 months prior to first dose of study intervention or active GVHD. 5. For participants with NDMM: Previous systemic treatment for MM except for a short course of corticosteroids (i.e., up to 4 days of 40 mg dexamethasone or equivalent before the first dose of study intervention). 6. Live attenuated vaccine administered within 4 weeks of the first dose of study intervention. 7. Administration of investigational product (e.g., drug or vaccine) concurrent with study intervention or within 30 days (or as determined by the local requirement) preceding the first dose of study intervention used in this study.

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5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Elranatamab administered in combination with daratumumab and lenalidomide, or in combination with lenalidomide. Part 1 will identify the optimal dose(s) of the combination regimen. Part 2 will utilize the identified optimal dose. Active Comparator arm will receive Daratumumab + Lenalidomide + Dexamethasone. **Route of Administration:** Subcutaneous (SC) for elranatamab and daratumumab; Oral for lenalidomide and dexamethasone. **Duration of Treatment:** 28-day cycles administered until disease progression or unacceptable toxicity.

5.2 Concomitant & Prohibited Therapy Permitted: Routine anti-infective prophylaxis is heavily recommended, including immunoglobulin replacement therapy, anti-viral prophylaxis, and anti-Pneumocystis jirovecii agents. **Prohibited:** Live attenuated vaccines administered within 4 weeks of the first dose, and concurrent use of other investigational products within 30 days.

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6. Study Timeline & Visit Schedule

| Visit ID | Window (Days) | Key Activities/Procedures | | :--- | :--- | :--- | | **Screening** | Day -28 to 0 | Informed Consent, IMWG criteria assessment, viral screening, baseline M-protein labs | | **Baseline** | Day 1 (Cycle 1) | Randomization, First Dose (Step-up dosing initiation for elranatamab), Safety Labs | | **Interim** | Day 1 of each Cycle | Efficacy Assessment (Serum/Urine M-protein), Safety monitoring (CRS/ICANS evaluation), PK/ADA labs | | **End of Study** | Follow-up | Final disease assessment, survival follow-up, and subsequent anti-myeloma therapy tracking |

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7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definitions: 1. Part 1: Dose Limiting Toxicity. 2. Part 2: Progression free survival by blinded independent central review. 3. Part 2: Minimal Residual Disease negativity rate. **Measurement Method:** Next-generation sequencing (NGS) or flow cytometry for MRD testing from bone marrow aspirates; standard IMWG criteria and blinded independent central review for PFS assessment.

7.2 Secondary & Exploratory Endpoints 1. Overall Survival. 2. Overall minimal residual disease negativity rate. 3. Sustained MRD negativity rate (Part 2).

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8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring for infections, cytopenias (neutropenia, anemia, thrombocytopenia), Cytokine Release Syndrome (CRS), and Immune Effector Cell-Associated Neurotoxicity Syndrome (ICANS). Part 1 specifically evaluates Dose Limiting Toxicities. **Serious Adverse Events (SAEs):** Must be reported within 24 hours of investigator awareness. **Data Monitoring Committee (DMC):** An external Independent Data Monitoring Committee (IDMC) periodically reviews unblinded safety and efficacy data.

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9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for all primary efficacy endpoints (MRD and PFS). Safety Set encompasses all participants who receive at least one dose of the study drug. **Sample Size Justification:** 966 patients, appropriately powered to detect statistically significant improvements in MRD negativity rates and PFS hazard ratios compared to the active control arm. **Handling of Missing Data:** Participants with missing data for MRD assessments will be considered MRD positive (non-responders) for the primary efficacy analysis.

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10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Regular onsite and remote monitoring visits to ensure protocol compliance, particularly regarding dosing administration and AE (CRS/ICANS) grading and management. **Audit Trail:** Validated eCRF systems with comprehensive audit trails, electronic signature capture, and data clarification mechanisms.

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11. Study Identifiers & Governance

Primary ID: PF-06863135 **Registry Number**: NCT05623020 **Sponsor/Lead Organization**: Pfizer **Study Title**: MagnetisMM-6 **Official Regulatory Title**: MAGNETISMM-6: AN OPEN-LABEL, 2-ARM, MULTICENTER, RANDOMIZED PHASE 3 STUDY TO EVALUATE THE EFFICACY AND SAFETY OF ELRANATAMAB (PF-06863135) + DARATUMUMAB + LENALIDOMIDE OR ELRANATAMAB + LENALIDOMIDE VERSUS DARATUMUMAB + LENALIDOMIDE + DEXAMETHASONE IN TRANSPLANT-INELIGIBLE PARTICIPANTS WITH NEWLY DIAGNOSED MULTIPLE MYELOMA

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12. Clinical Framework (Multiple Myeloma Specific)

Primary Condition: Multiple Myeloma **Diagnosis Threshold:** Active MM requiring systemic therapy per IMWG criteria (RRMM for Part 1, NDMM for Part 1/Part 2) **Medical Methodology:** Bispecific T-cell Engager + Monoclonal Antibody + IMiD combination therapy **Optimization Target:** Achievement of deep, sustained Minimal Residual Disease (MRD) negativity and prolonged Progression-Free Survival

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13. Study Procedures & Methodology

Management Pathway: Upfront T-cell redirection clinical pathway **Education Protocol (HCP):** Mandatory training on the identification, grading, and management of CRS and ICANS prior to site activation. **Education Protocol (Patient):** Education regarding infection prevention, CRS symptoms reporting, and adherence to prophylactic medications. **Quality Audit Mechanism:** Centralized laboratory review of bone marrow aspirates for precise MRD quantification and blinded independent central review for PFS.

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14. Observation & Follow-Up Schedule

Total Duration: Continuous until disease progression, unacceptable toxicity, or withdrawal of consent. **Onsite Visit Cadence:** Weekly during Cycle 1-2, bi-weekly through Cycle 6, and Day 1 of every 28-day cycle thereafter. **Remote Monitoring Frequency:** Continuous survival and subsequent therapy tracking post-progression. **Checklist Review Interval:** Regular cycle-by-cycle evaluations for response and toxicity.

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15. Sign-off & Approval

Role	Name	Signature	Date
Principal Investigator	[Name]	____	[DD-MMM-YYYY]
Clinical Operations Lead	[Name]	___	[DD-MMM-YYYY]
Lead Statistician	[Name]	_____	[DD-MMM-YYYY]